

Common Foot and Ankle Injuries and Conditions

First Metatarsal Joint Sprain (“Turf Toe”)

Injury overview	Capsuloligamentous sprain is typically caused by forced hyperextension of the first metatarsophalangeal (MTP) joint (great toe) and is usually combined with ankle dorsiflexion. Normal foot mechanics place significant stress on the first MTP joint. Activities such as running and jumping increase stress magnitude by 3 to 8 times body weight.
Surgical and rehabilitative management	Surgery is generally not required. Rest, including partial or non-weight bearing, and limiting the amount of toe extension typically decrease symptoms. Using stiff shoe insoles and supportive taping can provide passive stability throughout the healing process.
Return-to-play exercise and conditioning considerations	Use the results from movement quality assessments to guide exercise selection. Implement a progressive neuromuscular approach to strengthen intrinsic and extrinsic muscles in the foot, ankle, knee, and hip. Begin with bilateral exercises, progressing to single-leg functional activities.
Return-to-play criteria	Full range of motion (70 degrees of passive extension of first MTP joint) and single-leg balance efficiency. Athletes should be able to push off forcefully, tolerate changes in direction, and demonstrate good landing techniques during jumping or running activities.
Estimated timeline	2–8 weeks, depending on the initial degree of damage and related mechanical contributing factors.

Adductor Muscle Strain

Injury overview	Lateral sprain involving damage to the anterior talofibular ligament, calcaneofibular ligament, and posterior talofibular ligament. Medial sprains involve injury to the broad-oriented deltoid ligament. These ligaments together provide sensory feedback and overall joint stability for the ankle. Damage to these structures impairs the normal function of the ankle.
Surgical and rehabilitative management	In most cases, ankle sprains are managed nonsurgically. In more severe cases, in conjunction with bony fracture and syndesmosis disruption, surgical intervention is recommended. Active range-of-motion exercises usually begin early (2–5 days post-injury). Isolated resistance exercises are introduced as tolerated, and exercise intensity increases once full weight-bearing is restored.

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Return-to-play exercise and conditioning considerations	Restore normal gait and movement patterns without compensation. Stress the importance of the neuromuscular exercise continuum for progressions. Assessments include the star balance excursion test, shark skill test, 5–10–5, and T-drill.
Return-to-play criteria	Successful completion of a dynamic skill battery (based on specific sports needs) on back-to-back days without complications.
Estimated timeline	Nonsurgical: 2–6 weeks. Surgical: can exceed 8 weeks, depending on the scale of intervention performed.

Achilles Tendinopathy

Injury overview	Response to chronic overload to the protective sheath covering the Achilles tendon (most often, the actual tendon is not damaged).
Surgical and rehabilitative management	Rehabilitation efforts focus on eccentric strengthening of the gastrocnemius and soleus in multiplanar positions. Persistence of symptoms lasting 6–8 months could lead to surgical intervention.
Return-to-play exercise and conditioning considerations	Restore normal gait and movement patterns without compensation. Stress the importance of neuromuscular exercise continuum for progressions.
Return-to-play criteria	Assessments include the star balance excursion test, shark skill test, 5-10-5, T-drill, and triple hop for distance.
Estimated timeline	2 weeks to 6 months. Generally dependent on the amount of stress reduction (rest) allowed to optimize healing. Since this is self-limiting, athletes commonly continue to participate in their sport with modifications.

Plantar Fasciitis

Injury overview	Under normal conditions, the plantar fascia distributes forces in the foot while weight-bearing. Chronic overuse produces microtears that heal inadequately, leaving scar tissue that is resistant to stretching, and leading to stiff plantar fascia.
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Surgical and rehabilitative management	Surgical repair is typically not needed but can be helpful for chronic conditions that include the formation of bone spurs (calcitrant) on the heel. The use of a night splint can be an effective support device. It will position the foot in dorsiflexion during the sleeping hours, allowing the tissues to heal under a gentle stretch (as opposed to the typical plantar-flexed foot posture during sleeping).
Return-to-play exercise and conditioning considerations	Towel crunches, single-leg longitudinal balance, balance training exercises, and stretching of gastrocnemius/soleus complex.
Return-to-play criteria	Assessments include the star balance excursion test, shark skill test, 5-10-5, and T-drill.
Estimated timeline	2 weeks to 4 months.

Neuroma

Injury overview	Referred to as Morton's neuroma or plantar neuroma. This inflammation occurs in the neural tissue between the metatarsals, usually between the second and third metatarsal heads. Repetitive stress (running, jumping, tight-fitting shoes, high-heeled shoes) causes scar tissue formation around the nerve.
Surgical and rehabilitative management	Generally, not needed. Ensure proper footwear and fit.
Return-to-play exercise and conditioning considerations	Balance training continuum and landing mechanics education.
Return-to-play criteria	Assessments include the star balance excursion test, shark skill test, 5-10-5, and T-drill.
Estimated timeline	In most instances, this is a self-limiting ailment. Surgical cases will require 4–8 weeks of recovery.